A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Hanna El-Koury

Dear Dr El-Koury,

Re: Mr Domenico M Smundin
DOB: 1967-01-19
Address: 161 Melbourne Road, WILLIAMSTOWN, VIC, 3016

Summary

- **Biochemical remission achieved with faecal calprotectin of 27 mg/kg.**
- **Significant symptomatic improvement in bowel frequency and consistency on Q8W Vedolizumab therapy.**
- **For surveillance colonoscopy to assess mucosal healing and colorectal cancer risk.**
- **No action required by you.**

I had the pleasure of reviewing Domenico in the clinic today. He is currently experiencing a significant improvement in his gastrointestinal symptoms since commencing his current infusion therapy. He reports approximately four to five bowel movements per day, noting that his initial morning stools are now well-formed. This is a substantial progress from his previous baseline of more than five loose stools daily. He has only experienced one episode of faecal incontinence in the last six months, which occurred following a period of physical exertion.

Domenico describes a much-improved quality of life and reduced psychological distress regarding his bowel habit. He is now able to attend social events and work commitments without the constant anxiety of requiring urgent toilet access. While he still experiences occasional urgency—particularly if he ignores initial signals—he feels significantly more "solid" than he did during his previous clinical trial. Of note, he has gained approximately four kilograms, now weighing 84kg; I suspect this weight gain is a positive indicator reflecting the resolution of systemic inflammation and improved metabolic stability.

Impression and Plan Domenico has reached biochemical remission, with his faecal calprotectin dropping from over 1000 mg/kg to a current level of 27 mg/kg. This confirms that the current

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medication is highly effective at suppressing his colonic inflammation. As he has a long history of inflammation, he remains at an elevated risk for colorectal dysplasia. I have therefore recommended he undergo a surveillance colonoscopy in the coming weeks to formally assess for mucosal healing and to establish a surveillance interval, which will likely be every two years.

We will maintain his current treatment regimen. I have requested interval pathology and will review him again following his colonoscopy to discuss the findings.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Mr Michael Johnston

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065